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CS (MA) Rules, 1944

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CS (Medical Attendance) Rules, 1944

INTRODUCTION & STATUTORY BACKGROUND

The Central Services (Medical Attendance) Rules, 1944 — popularly known as the CS (MA) Rules, 1944 — were notified on 1st January, 1944. These Rules constitute the principal statutory framework governing free medical attendance and treatment to Central Government civil servants and their dependent family members in stations not covered by the Central Government Health Scheme (CGHS).

The Rules were made by the Governor General-in-Council in exercise of the powers conferred by sub-section (2) of Section 241 read with sub-section (3) of Section 313 of the Government of India Act, 1935. After the commencement of the Constitution of India, 1950, these Rules continue in force by virtue of Article 313 (Continuance in force of existing laws on conditions of service).

🔗 **EXAM TIP:** CS (MA) Rules, 1944 and CGHS together form a complementary scheme. CS (MA) Rules apply where CGHS is not in force; CGHS applies in 80 notified cities (being expanded to 100). For LDCE-IP, expect questions on (i) ward entitlement, (ii) family definition, (iii) reimbursement time-limits, (iv) treatment outside India, and (v) special diseases.

RULE 1 — SHORT TITLE AND EXTENT OF APPLICATION

(1) Short Title and Date of Notification

These rules may be called the Central Services (Medical Attendance) Rules, 1944. They were notified on 1st January, 1944.

(2) Categories to whom these Rules APPLY

- All Government servants whose conditions of service are prescribed by Rules made or deemed to be made by the Central Government, while on duty, leave or Foreign Service in India or under suspension (except those listed in sub-rule (3));
- Government servants on leave preparatory to retirement;
- Government pensioners on their re-employment;
- Defence Accounts Department personnel;

- Civilians paid from Defence Services Estimates;
- Central Government employees on terminal leave;
- Probationers;
- Apprentices in the whole-time service of Government;
- All State Government servants on deputation with the Central Government.

(3) Categories to whom these Rules DO NOT APPLY

- Persons in railway service (covered by Railway Services Medical Attendance Rules);
- Defence Services personnel (covered by ECHS / AFMS);
- Government servants on deputation abroad;
- Non-gazetted personnel stationed in or passing through Calcutta (legacy provision);
- Retired Government officials;
- Non-officials sent on deputation abroad;
- Government servants governed by CGHS while in CGHS-functioning stations;
- Officers of All India Services (covered by AIS (MA) Rules, 1954);
- India-based officers and staff serving in Missions abroad.



GOVERNMENT DECISIONS / CLARIFICATIONS

(1) All Government servants and members of their family governed by these Rules, while on tour or on leave visiting any place where CGHS is in force, may avail of CGHS facilities.

(2) Claims in respect of officials admitted before retirement but discharged after superannuation — the expenses shall be reimbursed by the official's own office (parent department).

RULE 2 — DEFINITIONS

(a) "**Authorized Medical Attendant**" (AMA) means a medical officer in the employ of the Central Government or a Private Medical Practitioner appointed/nominated by the Ministry/Department for providing medical attendance to its employees.

(b) "**Patient**" means a Government servant to whom these Rules apply and who has fallen ill.

(c) **"Treatment"** means the use of all medical and surgical facilities available at the Government hospital in which the Government servant is treated.



GOVERNMENT DECISIONS / CLARIFICATIONS

(1) Appointment of private registered medical practitioners as AMAs is to be done by the Head of Department concerned.

(2) No treatment shall be obtained from an AMA while he is on regular leave.

(3) Consultation / visiting / injection fees of AMA:

Category / Procedure	Prescribed Fee (Rs.)
I. Medical Post Graduate / Specialist — First consultation	100/-
I. Medical Post Graduate / Specialist — Subsequent consultation	60/-
II. Medical Graduate (MBBS or equivalent) — First consultation	70/-
II. Medical Graduate (MBBS or equivalent) — Subsequent consultation	40/-
III. Injection — Intramuscular / Subcutaneous	20/-
III. Injection — Intravenous	40/-

(4) Simultaneous treatment in two or more systems of medicine is permissible only for different ailments.

(5) Reimbursement of charges for clinical tests paid to private practitioners / institutions:

- Up to Rs. 500/- — a certificate from the medical officer in charge of the case in the Hospital that such facilities are not available in a Government Hospital is required.
- Exceeding Rs. 500/- — Prior approval of the Medical Superintendent of the Hospital is required.

(6) Ceiling for reimbursement of Special Nursing and Ayah / Attendant:

- Special Nurse: Rs. 150/- per shift of 12 hours.
- Ayah / Attendant: Rs. 75/- per shift of 12 hours.

(7) IVF (In-Vitro Fertilization) Treatment [OM No. Z.15025/5/2011-CGHS III/CGHS(P) dated 22.11.2011]:

- Age of woman undergoing IVF: between 21 and 39 years.
- The woman must be married and living with her husband.
- Permitted only in cases of infertility where the couple has no living issue.
- Reimbursement allowed up to a maximum of 3 (three) fresh cycles.
- Amount: not exceeding Rs. 65,000/- per cycle or actual cost, whichever is lower (inclusive of cost of drugs, disposables and monitoring).
- Prior approval mandatory; permission only on the recommendation of HOD Gynaecology & Obstetrics of a Government Medical Institution.

(8) Validity of AMA prescriptions:

- For tests / investigations at empanelled private hospitals on the recommendation of AMA — the prescription is valid for single use within 30 days from the date of prescription. Revalidation is required after expiry.
- Prescription / advice issued by AMA for any procedure is valid for three months.

(9) Stay in hospital — average stay as in-patient is reckoned as **two weeks**. Any stay beyond **three weeks** is considered as prolonged stay.

RULE 3 — MEDICAL ATTENDANCE

(1) A Government servant shall be entitled, free of charge, to medical attendance by the Authorized Medical Attendant.

(2) Where a Government servant is entitled under sub-rule (1), free of charge, to receive medical attendance, any amount paid by him on account of such medical attendance shall, on production of a certificate in writing by the AMA in this behalf, be reimbursed to him by the Central Government.

Note: If the controlling authority rejects the claim under any condition, the claimant may submit an appeal to the appropriate authority within a period of **forty-five days** from the date of receipt of the order rejecting the claim.

**GOVERNMENT DECISIONS / CLARIFICATIONS**

- (a) The first four consultations / visits should be completed within a period of ten days from the date of commencement of treatment in respect of one single and continuous spell of illness for a particular disease.
- (b) In Indian Systems of Medicine and Homoeopathy, the 10-day limit may be extended up to 20 days by the AMA.
- (c) A maximum of 15 (fifteen) injections may be administered for non-prolonged treatment at the consulting room of the AMA.

 **EXAM TIP:** If treatment exceeds 4 consultations OR 15 injections within the prescribed period, it qualifies as 'prolonged treatment' and the patient must be referred to OPD of a Government / recognized hospital — see Rule 6 Note (a).

RULE 4 — TRAVELLING ALLOWANCE FOR MEDICAL ATTENDANCE JOURNEYS

(1) When the place at which a patient falls ill is more than **five miles (8 km)** by the shortest route from the consulting room of the AMA —

- (a) the patient shall be entitled to travelling allowance for the journey to and from such consulting room; or
- (b) if the patient is too ill to travel, the AMA shall be entitled to travelling allowance for the journey to and from the place where the patient is.

(2) An application for travelling allowance under sub-rule (1) shall be accompanied by a certificate in writing by the AMA stating that medical attendance was necessary, and (if under clause (b)) that the patient was too ill to travel.

Travelling Allowance — Government Decisions

(1) The attendant will be entitled to travelling allowance both ways at the rate admissible to a member of the family of the Government servant.

(2) Kidney donor:

- (a) Donor is a private person — TA admissible at the rates applicable to the recipient Government servant.

- (b) Donor is another Government servant or his family member — TA admissible at the rates applicable to the donor Government servant.

(3) Special concession to TB and Cancer patients (rail travel concession for admission, discharge, re-examination or periodical check-up — under CS(MA) Rules / IRCA Coaching Tariff):

Sl. No.	For Whom Available	Nature of Concession (CS(MA) Rule)
1	Patient travelling accompanied by an attendant	Patient and attendant — ½ of normal journey fare
2	Patient travelling alone	Single journey ticket on ¼ of the normal fare

 **LEGISLATIVE NOTE:** The above rates remain the legacy CS(MA) Rule entitlement. However, separately under Indian Railways policy (IRCA Coaching Tariff Rule 101 read with Railway Board circulars), cancer patients now get 100% concession in Sleeper / 3AC, 75% in 2nd / SL / CC / FC, and 50% in 1AC / 2AC; TB patients (going to/from sanatorium) get 75% in SL / 2S. Suspension of senior-citizen concessions w.e.f. 20.03.2020 (Railway Board CC-22 of 2020) does NOT affect medical concessions, which continue to operate independently. [Ref: Railway Board CC-22/2020; IRCA Coaching Tariff Part-I, Vol-I]

RULE 5 — CONSULTATION WITH SPECIALIST

(1) If the AMA is of opinion that the case of a patient is of such a serious or special nature as to require medical attendance by some person other than himself, he may, with the approval of the Chief Administrative Medical Officer of the State (which shall be obtained beforehand unless the delay involved entails danger to the health of the patient) —

- (a) send the patient to the nearest specialist or other medical officer; or
- (b) if the patient is too ill to travel, summon such specialist or other medical officer to attend upon the patient.

(2) A patient sent under clause (a) of sub-rule (1) shall, on production of a certificate in writing by the AMA, be entitled to travelling allowance for journeys to and from the headquarters of the specialist.

(3) A specialist or other medical officer summoned under clause (b) shall, on production of a certificate in writing by the AMA, be entitled to travelling allowance for the journey to and from the place where the patient is.



GOVERNMENT DECISIONS / CLARIFICATIONS

(a) Testing of eyesight for glasses at recognized hospitals by specialists — once in every three years on the recommendation of the AMA. Fee paid to the specialist will be reimbursed.

RULE 6 — MEDICAL TREATMENT

(1) A Government servant shall be entitled, free of charge, to treatment —

- (a) in such Government hospital at or near the place where he falls ill as can, in the opinion of the AMA, provide the necessary and suitable treatment; or
- (b) if there is no such hospital as referred to in sub-clause (a), in such hospital other than a Government hospital at or near the place as can, in the opinion of the AMA, provide the necessary and suitable treatment.

(2) Where a Government servant is entitled under sub-rule (1), free of charge, to treatment in a hospital, any amount paid by him on account of such treatment shall, on production of a certificate in writing by the AMA, be reimbursed to him by the Central Government.

Note: Appeal against rejection of claim — within forty-five days (as in Rule 3).

Notes / Government Decisions

(a) Prolonged Treatment — Where treatment exceeds 4 consultations / visits within 10 days from the date of commencement of treatment, OR more than 15 injections, the patient should be referred to the OPD of a Government / recognized hospital.

(b) Treatment for Immunizing and Prophylactic Purposes — Reimbursable for treatment of self or family members in the case of communicable diseases only, viz.

Cholera, Typhoid group of fevers (TAB), Plague, Diphtheria, Whooping cough, Tetanus and Poliomyelitis.

(c) Vaccination for Hepatitis B is reimbursable.

 **LEGISLATIVE NOTE:** Pursuant to the COVID-19 pandemic and subsequent OMs of MoHFW, COVID-19 vaccination, testing (RT-PCR/RAT) and treatment have also been made admissible for reimbursement under CS (MA) Rules and CGHS for the period notified. [Ref: MoHFW OMs on COVID-19 reimbursement, 2020-2023]

RULE 7 — TREATMENT AT RESIDENCE

(1) If the AMA is of opinion that owing to the absence or remoteness of a suitable hospital or to the severity of the illness, a Government servant cannot be given treatment as provided in clause (a) of sub-rule (1) of Rule 6, the Government servant may receive treatment at his residence.

(2) A Government servant receiving treatment at his residence under sub-rule (1) shall be entitled to receive towards the cost of such treatment incurred by him a sum equivalent to the cost of such treatment as he would have been entitled, free of charge, to receive under these Rules if he had not been treated at his residence.

(3) Claims for sums admissible under sub-rule (2) shall be accompanied by a certificate in writing by the AMA stating —

- (a) his reasons for the opinion referred to in sub-rule (1); and
- (b) the cost of similar treatment referred to in sub-rule (2).

Note: If the AMA certifies that the Government servant required hospital treatment but no accommodation was available at the recognized hospital, the fees paid for medical treatment at the patient's residence may be reimbursed.

RULE 8 — OTHER MEDICAL FACILITIES

(1) Charges for services rendered in connection with, but not included in, medical attendance on or treatment of a patient entitled, free of charge, to medical attendance or treatment under these Rules, shall be determined by the AMA and paid by the patient.

(2) If any question arises as to whether any service is included in medical attendance or treatment, it shall be referred to the Government and the decision of the Government shall be final.

Notes / Government Decisions on Other Medical Facilities

- (a) Reimbursement of expenditure incurred on treatment of sterility is admissible.
- (b) Expenses incurred in connection with sterilization operation are refundable.
- (c) Expenses incurred by Central Government servants on Medical Termination of Pregnancy are reimbursable, provided the MTP has been performed at Government or other institutions / hospitals approved under the Medical Termination of Pregnancy Act, 1971 **UPDATED (as amended by the MTP (Amendment) Act, 2021)**

 **LEGISLATIVE NOTE:** MTP (Amendment) Act, 2021 (Act No. 8 of 2021) extended the upper limit for termination of pregnancy from 20 weeks to 24 weeks for special categories of women (rape survivors, minors, women with disabilities, etc.) and removed the requirement of opinion of one Registered Medical Practitioner for pregnancies up to 20 weeks. Reimbursement under CS (MA) Rules / CGHS now applies to MTPs performed in compliance with the amended Act.

- (d) Reimbursement of complete denture:
 - Complete denture — up to Rs. 2,000/-
 - Denture of one jaw — Rs. 1,000/-
- (e) Reimbursement of cost of artificial appliances (purchase / repair / replacement). Maintenance cost is to be borne by the beneficiary.
- (f) Re-issue of appliances after — 5 years for adults; 2 years for children (except motorized wheelchair and tricycle).
- (g) Motorized wheelchair and tricycle — re-issued after 5 years.
- (h) Prosthetic components and orthotic joints used in appliances shall have BIS / CE (European) certification for reimbursement and must be fabricated by firms employing qualified prosthetist / orthotists.
- (i) Up to age 12 years is considered the physical growth age.

(j) Items not in annexures: where item cost is below Rs. 50,000/-, may be purchased on three quotations with permission of HOD; where cost exceeds Rs. 50,000/-, prior permission of Additional Director, CGHS of the concerned city / DDG required, on three quotations and approval of Technical Standing Committee.

(k) The list of items and rates is to be revised every 5 years.

(l) Ceiling for reimbursement of digital hearing aid (one ear only). Replacement after 5 years on basis of condemnation certificate from technical expert / approval of CGHS / Government ENT surgeon:

Type of Hearing Aid	Ceiling (Rs.)
Body worn / Pocket type	3,000/-
Analogue BTE	7,000/-
Digital BTE	8,000/-
Digital ITC / CIC	9,000/-
LEGISLATIVE NOTE: Earlier ceiling for Digital BTE (Rs. 15,000) and Digital ITC/CIC (Rs. 20,000) prescribed by interim OM's were revised downward and finally settled at the rates above per OM No. Z.11011/37/2019-EHS. Empanelled hospitals must follow CGHS-revised rates effective 13.10.2025 (Tier-based).	

(m) Ceiling rates for Intra-Ocular Lenses (IOL):

Type of IOL	Ceiling (Rs.)
Hydrophobic Foldable IOL	5,800/-
Hydrophobic Acrylic IOL	4,500/-
PMMA IOL (AC / PC)	500/-

Type of IOL	Ceiling (Rs.)
Scleral Fixated IOL	3,500/-

(n) Ceiling for reimbursement of Coronary Stents — capped per NPPA notifications. Drug-Eluting Stents and Bare Metal Stents are subject to NPPA price ceilings as revised from time to time. Maximum of two (2) stents per patient or actual cost, whichever is lower. **UPDATED CGHS rates revised 13.10.2025**

LEGISLATIVE NOTE: Earlier ceiling of Rs. 60,000/- per stent has been superseded. As per NPPA price ceilings (DPCO 2013) and subsequent CGHS OM, the current ceiling rates for stents are linked to NPPA-notified retail prices. Beneficiaries should refer to the latest CGHS rate list at cghs.mohfw.gov.in for current rates applicable in their city tier. [Ref: NPPA Notifications; CGHS OM, F.No.Z15025/8/2023/DIR/CGHS dated 01.02.2024 and 13.10.2025]

(o) Ceiling for Bariatric Surgery — Rs. 2,25,000/- or actual, whichever is less.

(p) Ceiling for Cochlear Implant — Rs. 5,35,000/-.

(q) **Entitlement for Indoor Treatment** **UPDATED Revised w.e.f. 28.10.2022** — based on Basic Pay (in 7th CPC):

Basic Pay (per month) — 7th CPC	Ward Entitlement
Up to Rs. 36,500	General Ward
Rs. 36,501 to Rs. 50,500	Semi-Private Ward
Above Rs. 50,500	Private Ward

LEGISLATIVE NOTE: Substituted for the earlier slab (Up to Rs. 47,600 / Rs. 47,601-63,100 / Rs. 63,101 & above) vide MoHFW OM No. S.11011/11/2016-CGHS(P)/EHS dated 28.10.2022 in partial modification of OM dated 09.01.2017. Clarification issued vide MoHFW OM dated 22.11.2022. The same revised slab

Basic Pay (per month) — 7th CPC**Ward Entitlement**

applies to Ward Entitlement under Item (4) of CGHS section below. [Ref: MoHFW OM dated 28.10.2022 & 22.11.2022; Lok Sabha Unstarred Q. No. 3871 dated 11.08.2023]

(r) Annual Medical Examination for Group 'A' officers of Central Civil Services aged 40 years and above:

- Men officers: Rs. 2,000/- or actual, whichever is less.
- Women officers: Rs. 2,200/- or actual, whichever is less.

RULE 9 — COUNTERSIGNATURE OF CERTIFICATES

The Controlling Officer of a patient may require that any certificate required by these Rules to be given by the AMA for travelling allowance purposes shall be countersigned —

- (a) in the case of a certificate given by the Principal Medical Officer of a District — by the Chief Administrative Medical Officer of the State; and
- (b) in the case of a certificate given by any other medical officer — by the Principal Medical Officer of the District.

RULE 10 — TRANSFER TO FOREIGN SERVICE

No Government servant shall be transferred to foreign service unless the foreign employer undertakes to afford to him, so far as may be, privileges not inferior to those which he would have enjoyed under these Rules if he had been employed in the service of the Government of India.

RULE 11 — TREATMENT OUTSIDE INDIA

(1) A Government servant shall be eligible to obtain medical treatment outside India, or to claim reimbursement of the cost of medical treatment obtained inside or outside India, in accordance with the provisions of this rule.

(2) A Government servant desirous of availing of medical treatment outside India may make an application through the Department / Ministry to which he is attached, to the Standing Committee established under this rule, in the form specified by the Standing Committee.

(3) A Government servant desiring to avail medical treatment outside India for himself or for a member of his family for any treatment specified in the Table below shall, subject to the other provisions of this rule, be eligible:

TABLE – Treatments Eligible for Outside India

- (1) Complex / high risk Cardio-Vascular Surgery cases for treatment at Centres with extensive experience;
- (2) Bone marrow Transplant;
- (3) Complex Medical and Oncological Disorders, such as leukemia and Neoplastic conditions;
- (4) Complex high risk cases in Micro-Vascular and Neuro Surgery for treatment at Centres with extensive experience;
- (5) Treatment of extremely complex ailments (other than the above) which, in the opinion of the Standing Committee, can only be treated abroad and fall in the high-risk category.

(4) The Central Government may from time to time review the list of treatment facilities specified in the Table to sub-rule (3) and make additions or deletions by notification in the Official Gazette.

(5) The Central Government may, for the purposes of this rule, constitute a Standing Committee consisting of:

- (a) Director-General of Health Services, Ministry of Health, Central Government;
- (b) Director-General of Armed Forces Medical Services;
- (c) Director-General of the Indian Council of Medical Research; and
- (d) Joint Secretary in the Ministry of Health and Family Welfare (Convener).

for purposes of considering and recommending to the Central Government cases for medical treatment outside India.

(6) On receipt of an application, the Standing Committee may, after due consideration and on being satisfied that the ailment / treatment can be treated only outside India, issue a certificate of approval.

(7) The Central Government may authorize reimbursement of expenditure on medical treatment obtained outside India where prior approval could not be obtained due to circumstances beyond control, provided all other conditions are fulfilled.

(8) The Standing Committee may, in the interest of the Government servant or family member, recommend one attendant to accompany; expenditure incurred is also reimbursable.

(9) Where adequate facility is available within India, the Standing Committee shall record such finding and authorize treatment in such Indian institution; cost shall be reimbursed.

(10) For purposes of sub-rule (9), the Ministry of Health, in consultation with the Standing Committee, shall from time to time notify names of such institutions along with ailments / treatments available.

(11) The scale of expenditure and eligibility for treatment shall be identical to that of an officer of the Indian Foreign Service of corresponding grade in the Ministry of External Affairs under the Assisted Medical Attendance Scheme in force.

GENERAL GOVERNMENT DECISIONS / PROCEDURAL MATTERS

1. Procedural Decisions on Medical Claims

- (1) Duplicate cash memos in case of loss must be duly countersigned by the AMA.
- (2) Cash memoranda attached to medical claims may be required to be cancelled to prevent dual claims.
- (3) Designation and degree of the doctor must be indicated on the claim papers.
- (4) **Time-limit for medical claims submission** **UPDATED Standardized to 6 months :**

Category of Claim	Time-limit
Completion of out-door treatment	6 months
Discharge from hospital (in-patient)	6 months
Adjustment of advance after discharge	1 month

LEGISLATIVE NOTE: Substituted for earlier '3 months' time-limit vide MoHFW OM No. S.14025/19/2015-MS/CGHS(P) and subsequent orders standardizing the time limit to 6 months for all out-patient and in-patient claims.

Category of Claim	Time-limit
Updated time-limits apply to all serving employees, pensioners and family pensioners.	

(5) Approval limit of Medical Reimbursement in emergency: there is no financial limit; up to Rs. 2 lakhs — Head of Office (not below the rank of Joint Secretary); above Rs. 2 lakhs — IFA (Integrated Financial Adviser).

2. Grant of Medical Advance

(1) Ordinarily, not more than one advance shall be granted in respect of the same illness or injury.

(2) Advance limit:

- (a) Rs. 10,000/- — for indoor treatment in hospital and out-patient treatment for illnesses such as cancer, where the duration of treatment is 3 months or less.
- (b) 80% of estimated cost or Rs. 36,000/-, whichever is less — for TB where duration of treatment exceeds 3 months.
- (c) 90% of the package deal (wherever it exists) — for major illnesses such as Bypass surgery, Kidney transplant, major cancer treatment, etc.

(3) The advance is paid directly to the hospital concerned on receipt of an estimate from the treating Physician / Medical Superintendent of the Hospital.

3. Treatment of Special Diseases

(1) Mental Diseases **UPDATED Now governed by Mental Healthcare Act, 2017** — duration of treatment may be extended for a maximum period of three years.

(2) Treatment should be allowed initially for six months, further extendable in blocks of six months, in mental diseases.

 **LEGISLATIVE NOTE:** The Mental Healthcare Act, 2017 (Act No. 10 of 2017), w.e.f. 29.05.2018, replaced the Mental Health Act, 1987. Section 21 of the 2017 Act guarantees right to access mental healthcare, including treatment from services run / funded by Government, with right to community living. Treatment under CS(MA) Rules / CGHS for mental illness is now to be read with the 2017 Act, MHCA Rules 2018 and the Rights of Persons with Disabilities Act, 2016.

(3) Tuberculosis: maximum period of treatment from the date of detection for purposes of reimbursement shall be restricted to 24 months as certified by the TB specialist / Medical Attendant.

(4) Cancer: treatment can be availed in any recognized cancer institute on the recommendation of the AMA.

(5) Post-treatment check-up suggested is also admissible without recommendation of the AMA.

(6) Diabetes: cost of medicines may be reimbursed even for a period beyond three months on the advice of the AMA.

4. Concession for Families

(1) The family of a Government servant is entitled to receive medical attendance and treatment at the hospital at which the Government servant himself is entitled.

(2) **"Family" for the purposes of these Rules** means: wife or husband, parents, widowed sisters, widowed daughters, minor brothers, children, step-children, separated / divorced daughters, and step-mother — wholly dependent upon the Government servant, normally residing with the Government servant or, in the case of parents, at another station.

(3) Age limit of dependents:

Sl. No.	Member	Age Limit / Conditions
1	Son / adopted legal child	Until he starts earning OR attains 25 years OR gets married, whichever is earlier.
2	Daughter	Until she starts earning OR gets married — irrespective of age limit, whichever is earlier.
3	Son with permanent disability (physical / mental)	Irrespective of age limit.

Sl. No.	Member	Age Limit / Conditions
4	Dependent divorced / abandoned / separated / widowed daughters AND dependent unmarried / divorced / abandoned / separated / widowed sisters	Irrespective of age limit.
5	Minor brother(s)	Up to attaining majority.
6	Permanently disabled brother(s)	Irrespective of age limit.

(4) Women's hospitals are admissible for male children up to 6 years.

(5) A member of the family is treated as a dependent only if his/her income from all sources (including pension and family pension) is less than Rs. 9,000/- plus DA thereon.

UPDATED Per OM No. 04/02/2014-P&PW(D), this minimum dependency income is currently Rs. 9,000 + DA

- (a) The term 'children' includes children adopted legally.
- (b) The term 'wife' includes more than one wife (where lawfully recognized).
- (c) Parents may reside either with the Government servant or with the family members at another station.

(6) When both husband and wife are Government servants, both can claim reimbursement of medical expenses for their respective parents.

(7) Option to include parents OR parents-in-law **UPDATED Now available to BOTH male & female employees** — Earlier this option was available only to female Government employees once during entire service. As per OM dated 26.10.2017, both male and female Central Government employees may exercise an option (one-time) to include either their own parents or parents-in-law as CGHS / CS(MA) beneficiaries.

LEGISLATIVE NOTE: The earlier discriminatory provision allowing only female employees to substitute parents-in-law has been corrected. Vide MoHFW OM No. S.11011/4/2017-CGHS(P)/EHS dated 26.10.2017, male Government

employees are also eligible to exercise this one-time option in line with the principle of equality. [Ref: MoHFW OM dated 26.10.2017]

(8) The restriction of two children is NOT applicable for claiming medical reimbursement.

(9) A Government employee under suspension is entitled for reimbursement of all medical expenditures at par with working employees.

(10) **"Disability"** **UPDATED Now defined under RPwD Act, 2016** — The earlier list of 7 disabilities (under PwD Act, 1995) has been replaced by the list of 21 disabilities recognized under the Rights of Persons with Disabilities Act, 2016 (effective 19.04.2017):

Group	Specified Disabilities (RPwD Act, 2016)	Examples / Definitions
Physical Disabilities	1. Locomotor Disability 2. Leprosy Cured Persons 3. Cerebral Palsy 4. Dwarfism 5. Muscular Dystrophy 6. Acid Attack Victims	Loss of mobility; deformity post-leprosy; non-progressive neurological condition; height $\leq 4'10''$; hereditary muscle disease.
Visual Disabilities	7. Blindness 8. Low Vision	As certified per RPwD Rules, 2017.
Hearing Disabilities	9. Deaf 10. Hard of Hearing	Hearing loss ≥ 70 dB / ≥ 60 dB to ≥ 70 dB.
Speech & Language	11. Speech and Language Disability	Permanent disability arising out of laryngectomy, aphasia, etc.
Intellectual Disabilities	12. Intellectual Disability (incl. Specific Learning Disability) 13. Autism Spectrum Disorder	Significant limitations in intellectual functioning and adaptive behaviour.

Group	Specified Disabilities (RPwD Act, 2016)	Examples / Definitions
Mental Behaviour	14. Mental Illness	Substantial disorder of thinking, mood, perception (excludes mental retardation).
Disability — Chronic Neurological	15. Multiple Sclerosis 16. Parkinson's Disease	Progressive degenerative neurological conditions.
Blood Disorders	17. Haemophilia 18. Thalassemia 19. Sickle Cell Disease	Hereditary blood disorders.
Multiple Disabilities	20. Multiple Disabilities including Deafblindness 21. Any other category notified by Central Government	Combination of two or more specified disabilities.

 **LEGISLATIVE NOTE:** The Rights of Persons with Disabilities (RPwD) Act, 2016 (Act No. 49 of 2016) replaced the Persons with Disabilities (Equal Opportunities, Protection of Rights and Full Participation) Act, 1995. The Act came into force on 19.04.2017 vide MoSJ&E notification. The number of recognized disability categories has been increased from 7 (under the 1995 Act) to 21 (under the 2016 Act). Reservation in Government employment for persons with benchmark disabilities increased from 3% to 4%. [Ref: RPwD Act, 2016; RPwD Rules, 2017; MoSJ&E Notification dated 04.01.2018]

(11) "Confinement, pre-natal and post-natal treatment" means treatment received by the wife of a Government servant before and after child-birth for physiological or other disability attributable to child-birth.

CENTRAL GOVERNMENT HEALTH SCHEME (CGHS)**CENTRAL GOVERNMENT HEALTH SCHEME (CGHS)****(1) CGHS — Coverage and Cities** **UPDATED Updated 2025-26**

Presently approximately 47.44 lakh (~4.74 million) beneficiaries are covered under CGHS in 80 cities all over India. The Government has approved expansion to 100 cities in a phased manner. CGHS was established on 1st July, 1954 (originally as the Contributory Health Service Scheme in Delhi) and currently functions under the Ministry of Health & Family Welfare.

 **LEGISLATIVE NOTE:** Substituted for the earlier figures of "45 lakh beneficiaries" and "85 cities". Latest figures as per official sources (cghs.mohfw.gov.in and Wikipedia, 2025-26): ~47.44 lakh beneficiaries; 80 functional cities; ~2,486 empanelled private hospitals and diagnostic centres. Expansion plan to 100 cities is being implemented by MoHFW. The CGHS Headquarters is at CGHS Bhawan, R.K. Puram, New Delhi.

(2) CGHS provides healthcare through the following systems of medicine:

- (a) Allopathic
- (b) Homoeopathic
- (c) Indian Systems of Medicine — (i) Ayurveda; (ii) Unani; (iii) Siddha; (iv) Yoga

(3) Eligibility:

- (a) All Central Government employees paid from the Central Civil Estimates (except Railways and Delhi Administration), including their dependent family members residing in CGHS-covered areas.
- (b) Pensioners of Central Government (except those of Railways and the Armed Forces) and their families.
- (c) Widows of Central Government pensioners in receipt of family pension.
- (d) Members of Parliament (sitting and ex-MPs), Judges of Supreme Court & High Courts, Freedom Fighters, employees of certain autonomous bodies, Railway Board employees, accredited Journalists (PIB) and other notified categories.

(4) CGHS Contribution & Ward Entitlement (7th CPC) **UPDATED Ward Entitlement revised w.e.f. 28.10.2022**

For serving Central Government employees residing in CGHS-covered areas, obtaining a CGHS card is compulsory. Monthly contribution (deducted from salary) per the 7th Pay Commission (effective 1st February, 2017):

Level in Pay Matrix (7th CPC)	Contribution per month (Rs.)
Level 1 — 5	250
Level 6	450
Level 7 — 11	650
Level 12 and above	1,000

 **LEGISLATIVE NOTE:** Effective date corrected from "1.1.2017" to "1.2.2017" — As per MoHFW OM dated 09.01.2017 and modification dated 13.01.2017, the revised CGHS subscription rates became effective from 1st February, 2017 (instead of 1st January, 2017).

Pensioners / family pensioners contribute the amount they were subscribing at the time of retirement / death of the Government servant. They may pay either on yearly basis or one-time (10 years = 120 months) for life-long validity of the pensioner CGHS card.

Ward Entitlement in CGHS-Empanelled Private Hospitals — REVISED

Basic Pay (per month) — 7th CPC	Ward Entitlement
Up to Rs. 36,500	General Ward
Rs. 36,501 to Rs. 50,500	Semi-Private Ward
Above Rs. 50,500	Private Ward

Basic Pay (per month) — 7th CPC	Ward Entitlement
 LEGISLATIVE NOTE: Substituted for the earlier slabs (General up to Rs. 47,600; Semi-Pvt Rs. 47,601 — 63,100; Pvt Rs. 63,101 & above) vide MoHFW OM No. S.11011/11/2016-CGHS(P)/EHS dated 28.10.2022 — in partial modification of OM dated 09.01.2017. Clarification issued vide OM dated 22.11.2022. CGHS beneficiaries (serving and pensioners) eligible for upgradation may apply for fresh CGHS cards. [Ref: MoHFW OMs dated 28.10.2022 and 22.11.2022; Lok Sabha Unstarred Q. No. 3871 dated 11.08.2023]	
 EXAM TIP: THIS IS THE MOST FREQUENTLY QUOTED OUTDATED FIGURE in older study materials. For LDCE-IP, REMEMBER: General — up to Rs. 36,500 / Semi-Pvt — Rs. 36,501 to Rs. 50,500 / Private — above Rs. 50,500 (vide OM dated 28.10.2022).	

(5) CGHS Card Colour Scheme (Plastic cards now supplemented by e-CGHS cards):

Beneficiary Category	Card Colour
Serving Government employees	Blue
Pensioners, Ex-MPs, Freedom Fighters, etc.	Green
Members of Parliament (sitting)	Red
Beneficiaries of Autonomous Bodies / Journalists	Yellow

 **LEGISLATIVE NOTE:** CGHS has progressively migrated to digital e-CGHS cards (downloadable through the CGHS beneficiary portal mybeneficiary.cghs.gov.in) with Aadhaar / ABHA ID linkage. Plastic cards continue to be issued, but e-cards are valid for all CGHS purposes. Aadhaar-based

Beneficiary Category	Card Colour
authentication and ABHA ID linkage are being implemented as part of the digital health framework.	

(6) Plastic Duplicate CGHS card may be obtained against a lost card by applying to the concerned Additional Director along with a copy of FIR, on payment of Rs. 50/-.

(7) Fixed Medical Allowance (FMA) **UPDATED** Currently Rs. 1,000/- p.m.; FMA extended to NPS pensioners w.e.f. 07.02.2025

FMA is granted to Central Government Pensioners / Family Pensioners residing in areas not covered under CGHS, currently at Rs. 1,000/- per month (w.e.f. 01.07.2017). FMA was first introduced at Rs. 100 (1997), revised to Rs. 300 (2008), Rs. 500 (2014) and Rs. 1,000 (2017).

LEGISLATIVE NOTE: (i) Vide DoP&PW OM No. 04/07/2020-P&PW(D) dated 07.02.2025, FMA at Rs. 1,000/- per month has been formally extended to Central Government employees retired under the National Pension System (NPS), released on a quarterly basis through the bank, subject to submission of life certificate. (ii) A proposal to enhance FMA from Rs. 1,000 to Rs. 3,000 per month (per DRPSC recommendations) is currently part of the 8th CPC Terms of Reference but has NOT yet been implemented as on May 2026. [Ref: DoP&PW OM dated 07.02.2025; Lok Sabha Reply dated 06.12.2024; SCOVA 34th Meeting Minutes dated 11.03.2025]

(8) A serving employee can apply for a pensioner CGHS card along with pension papers. Application with enclosures and bank draft is to be forwarded to CGHS through the office. The pensioner card is issued on the day of retirement (provided applied for at least 6 weeks before retirement) and gets activated from the next day.

(9) Where both husband and wife are employed under Central Government, they are NOT eligible for separate CGHS cards. The spouse drawing higher pay shall make the card; the other spouse is eligible to avail CGHS benefits as a member.

(10) The service card is valid till the date of retirement, if otherwise the employee is eligible.

(11) On transfer of a serving employee to a non-CGHS-covered area, the service card remains valid for the family members up to 6 months after transfer, provided CGHS contribution for 6 months is paid in advance.

(12) On transfer of a serving CGHS beneficiary from one department / ministry to another in the same city, the same CGHS card continues. The new department / ministry shall duly inform CGHS for necessary database updation.

(13) Supply of Medicines from Wellness Centres:

- (a) Medicines for chronic diseases may be issued for 3 months at a time by any CGHS Doctor against a valid prescription of a specialist.
- (b) If the beneficiary is planning to go abroad, medicines may be issued up to 6 months at a time with permission of the Additional Director (with copy of valid prescription, CGHS card and proof of travel).

(14) Hospitalization in Emergency:

- (a) In emergency, a CGHS beneficiary may be admitted to any Government / private hospital. Reimbursement is at CGHS rates. Full reimbursement may be allowed in specific cases on the recommendation of the Technical Standing Committee (vide OM No. 4-18/2005-C&P-[Vol.I (Pt 1)] dated 20.02.2009).
- (b) The private hospital must issue an emergency certificate stating in detail the condition of the patient that justified emergency.
- (c) CGHS-empanelled hospitals CANNOT refuse admission in emergency to a CGHS beneficiary and must provide treatment on credit to pensioner beneficiaries and other entitled categories (even if the beneficiary is from a different CGHS city / outside CGHS city).

(15) For treatment in emergency at a non-empanelled hospital, OR expenditure incurred at an empanelled hospital, a Medical Reimbursement Claim (MRC) must be submitted by the beneficiary.

(16) **MRC submission time-limit** **UPDATED 6 months** — The claim must be submitted to the concerned department by serving employees and by pensioner beneficiaries within 6 months of discharge from hospital.

 **LEGISLATIVE NOTE:** Substituted for the earlier '3 months' limit vide MoHFW OM No. S.14025/19/2015-MS/CGHS(P) and subsequent orders. The time-limit has been standardized to 6 months for all medical claims (out-patient, in-patient and follow-up).

(17) Empanelled Hospitals & Diagnostic Centres **UPDATED Updated 2025-26**

Approximately 2,486 private hospitals and diagnostic centres are empanelled under CGHS as on 2025-26. In 2025 alone, a record 468 hospitals were freshly empanelled. All existing Memoranda of Agreement (MoAs) ceased to be valid w.e.f. 13.10.2025; hospitals were required to obtain fresh empanelment within 90 days under the revised package rates.

 **LEGISLATIVE NOTE:** Substituted for the earlier figures "1,303 empanelled hospitals and 206 diagnostic centres as on 15.06.2020". Latest figures as per CGHS data and MoHFW reports: ~2,486 total empanelled HCOs (Health Care Organizations); the network has expanded materially under the 2025 reforms. [Ref: CGHS Empanelment Network Expansion Report 2025; MoHFW Press Releases]

(18) Monetary ceiling for free diet for CGHS beneficiaries — pay / pension / family pension of Rs. 44,900/- per month.

(19) Monetary ceiling for free diet for beneficiaries suffering from TB or Mental disease — pay / pension / family pension of Rs. 69,700/- per month.

(20) Any pensioner / family pensioner entitled to avail CGHS facility — the rate of contribution shall be determined with reference to the level of pay he / she would have drawn in the post held at the time of retirement / death, had he / she continued in service now but for retirement / death.

5. Major CGHS Reforms — Effective 13.10.2025

NEW New 2025 Reform — Important for LDCE — The Union Ministry of Health & Family Welfare announced a comprehensive overhaul of CGHS package rates and empanelment framework, effective 13.10.2025. This was the first major rate revision since 2014.

(a) Tier-Based City Classification

CGHS cities are now classified into Tier I, Tier II and Tier III for differential package rate purposes. North-Eastern States and the UTs of Jammu & Kashmir and Ladakh follow Tier II rates.

(b) Accreditation-Based Differential Rates

Hospital Category	Rate Differential
NABH / NABL accredited hospital	Standard CGHS rates
Non-NABH / Non-NABL hospital	15% reduction from standard rates
Super-specialty hospital	15% premium over accredited rates

(c) Multiple Procedure Reimbursement Rule

In a single session: first procedure reimbursed at 100%, second at 50%, each subsequent procedure at 25%. Bilateral or identical procedures on different sites — full rate for one side and 50% for the second.

(d) Re-Empanelment Mandate

All existing MoAs with CGHS-empanelled private hospitals stood terminated at 12:00 AM on 13.10.2025. Hospitals were required to submit undertakings accepting the new MoA terms by that date and to execute fresh MoAs within 90 days through the Hospital Engagement Module on the CGHS portal. Hospitals failing to comply were to be de-panelled.

 **LEGISLATIVE NOTE:** These reforms are part of MoHFW's modernization drive. Cashless treatment is now also available at AIIMS New Delhi, PGIMER Chandigarh and JIPMER Puducherry. CGHS rates for nearly 2,000 medical procedures have been revised — the first comprehensive overhaul in over a decade. Beneficiaries should refer to Annexure-I of the official CGHS website (cghs.mohfw.gov.in) for the full revised rate list applicable in their city tier. [Ref: MoHFW CGHS Rate Revision OM dated October 2025; CGHS website annexures]

6. Recent Pay Commission & DA-Linked Updates

NEW Updated April-May 2026 — As of May 2026, the following DA / pay-related developments affect CGHS subscription and FMA quantum (indirect impact via pay slabs):

- DA / DR for Central Government employees and pensioners hiked to 60% from 1st January, 2026 (vide MoF OM dated April 2026).
- 8th Central Pay Commission constituted in 2025-26; Terms of Reference include re-examination of CGHS subscription and FMA quantum.
- 8th CPC has invited applications for consultant positions and is in the process of receiving memoranda from JCM (Staff Side) and pensioner associations.



QUICK REVISION CAPSULE FOR LDCE-IP / PS GROUP B

KEY POINTS – RAPID REVISION

Item / Provision	Quick Reference
Notification of CS(MA) Rules	1st January, 1944
Constitutional basis (post-1950)	Article 313 (continuance in force)
Distance threshold for TA	5 miles / 8 km from AMA's consulting room
Maximum consultations / visits in 10 days	4 (Allopathy); 20 days (Indian Medicine / Homoeopathy)
Maximum injections (non-prolonged)	15
Average hospital stay (in-patient)	2 weeks; >3 weeks = prolonged
Appeal time-limit (claim rejection)	45 days
MRC submission time-limit (NEW)	6 months (was 3 months)
AMA prescription validity (procedure)	3 months
AMA prescription single-use validity	30 days
IVF age range / cycles / cap	21–39 years / 3 cycles / Rs. 65,000 per cycle
Eyesight test (glasses)	Once in 3 years
Cochlear implant ceiling	Rs. 5,35,000/-
Bariatric surgery ceiling	Rs. 2,25,000/-
Coronary stents (max per patient)	2 stents (rates per NPPA / CGHS-revised)
Special Nurse fee	Rs. 150/- per 12-hour shift

Item / Provision	Quick Reference
Ayah / Attendant fee	Rs. 75/- per 12-hour shift
FMA (current)	Rs. 1,000/- per month
Free diet income ceiling (general)	Rs. 44,900/- p.m.
Free diet income ceiling (TB/Mental)	Rs. 69,700/- p.m.
CGHS cities (current)	80 (expanding to 100)
CGHS beneficiaries (approx.)	47.44 lakh (~4.74 million)
RPwD Act, 2016 — total disabilities	21 (was 7 under PwD Act, 1995)
MTP Act amendment	MTP (Amendment) Act, 2021 (gestation extended to 24 weeks)
Mental Healthcare Act	Mental Healthcare Act, 2017 (replaced 1987 Act)
Standing Committee for Treatment Outside India	DGHS + DG-AFMS + DG-ICMR + JS-MoHFW (Convener)

Key OMs / Notifications to Remember

S. No.	Reference / Date	Subject
1	MoHFW OM dated 09.01.2017 (Pay Slab) — w.e.f. 01.02.2017	CGHS Subscription per 7th CPC Pay Matrix Levels
2	MoHFW OM No. S.11011/11/2016-CGHS(P)/EHS dated 28.10.2022	REVISED Ward Entitlement (36,500 / 50,500 slabs)
3	MoHFW OM dated 22.11.2022	Clarification on Ward Entitlement revision

S. No.	Reference / Date	Subject
4	MoHFW OM dated 26.10.2017	Male employees also eligible for parents-in-law option
5	DoP&PW OM No. 04/07/2020-P&PW(D) dated 07.02.2025	FMA extended to NPS pensioners
6	MoHFW CGHS Rate Revision (October 2025)	Tier-based package rates effective 13.10.2025
7	Mental Healthcare Act, 2017 (No. 10 of 2017)	Replaced Mental Health Act, 1987
8	RPwD Act, 2016 (No. 49 of 2016)	21 disabilities; 4% reservation
9	MTP (Amendment) Act, 2021 (No. 8 of 2021)	Gestation period extended to 24 weeks
10	OM No. S.14025/19/2015-MS/CGHS(P)	MRC time-limit standardized to 6 months

🔗 **EXAM TIP:** LDCE-IP question patterns: (i) Match the OM with the subject; (ii) State the revised slab / ceiling; (iii) Differentiate between PwD Act 1995 and RPwD Act 2016; (iv) Identify what is NOT applicable to CS(MA) Rules; (v) Steps for emergency hospitalization in non-empanelled hospital.

Final Note for Aspirants

This document is a study aid only. For LDCE-IP, PS Group B and other competitive postal departmental examinations, candidates are advised to also refer to the original Office Memoranda available on cghs.mohfw.gov.in, the Postal Manual Volume V (where

applicable), and the latest Compendium of Establishment Orders. Whenever in doubt, the language of the latest OM / Gazette notification prevails.

== End of CCS (Medical Attendance) Rules, 1944 — Updated Edition ==

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